Thanks so much to the South Metro Human Services team of Minnesota for sharing their stellar program manual with us in the State of Missouri! This is a terrific exemplar for teams to work from in developing their own program manual and forms.

South Metro Human Services

Dialectical Behavior Therapy Clinic

Program Manual

April 10, 2011

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Description of the Agency

South Metro Human Services is a non-profit agency providing case management, behavior/chemical health, supportive housing and homeless services in Anoka, Carver, Dakota, Hennepin, Ramsey and Washington counties. The agency has approximately 2,200 active clients and employs over 300 mental health professionals, practitioners and support staff.

There are five clinical divisions and three operational divisions. The clinical divisions include Case Management, Residential and Supportive Housing, Homeless Services, Behavior Health, and the Training Institute. The operational divisions include Human Resources, Information Technology, and Finance.

I: Case Management Division:

A. Care Coordination – SMHS provides care coordination between behavioral health and primary care participants in *Ucare Connect* in the 7 county metro area.
B. Targeted Case Management (TCM) – SMHS provides TCM services to approximately 900 serious and persistently mentally ill clients in Ramsey County.
C. Assertive Community Treatment (ACT) – The agency operates 4 teams serving 380 clients in Anoka, Hennepin, and Ramsey County. Each ACT team is comprised of a psychiatrist, a mental health professional, two Registered Nurses, two supportive employment specialists, and five mental health/substance abuse practitioners.
D. Adult Rehabilitative Mental Health Services (ARMHS) – This program provides services to mentally ill clients or clients with a co-occurring disorder living in the community in Anoka, Ramsey, and Hennepin Counties.
E. Community Alternatives for Disabled Individuals (CADI) – This program assists individuals who are currently in an institutional move to the community with appropriate support services in Ramsey County.
F. Traumatic Brain Injury (TBI) - Case Management in Ramsey County.
G. Pre-petition Screening – Staff screen clients under MS 253B (civil commitment in Ramsey County).

II: Residential and Supportive Housing

A. Corporate Adult Foster Care (AFC): The agency operates 15 AFC facilities in Anoka, Dakota and Ramsey County.
B. Community Foundations – Intensive Residential Treatment Services (IRTS) - This is a 16-bed facility located in Ramsey County that serves as an alternative to acute care hospitalization. Clients are typically admitted for less than 90 days for stabilization and are discharged back to the community.
C. SMHS Public Housing Agency (PHA) – SMHS is a Public Housing Agency under HUD and administers 75 Mainstream Section 8 Vouchers in Hennepin and Ramsey County.
D. The Rose Center: This is a seven-unit apartment complex providing rental assistance and support services to vulnerable adult women and their children.

E. Crane Ordway – This is fourteen apartment units with rental assistance and support services for adult mentally ill persons or persons with co-occurring mental illness and substance use disorder.
F. Extended Care apartments – Approximately 35 units of scattered site rental assistance and ARMHS support.
G. P-Hop – Provides approximately 32 units of rental assistance and support services to homeless individuals
H. Main Street Housing Partnership – A joint program with Catholic Charities and Mary Hall providing 40 SRO units of transitional rental assistance and supportive services to homeless mentally ill person and or persons with co-occurring disorders.
I. Saint Paul Residence - A joint program with Catholic Charities and Mary Hall providing 17 units of permanent SRO housing and support services.

III: Homeless Services –

A. ACCESS – This is a PATH (federal) program providing outreach and case management to homeless individuals with mental illness.
B. SOAR – This program assists individuals applying for SSA benefits. A psychologist provides Consultative Evaluations for the medical record and a mental health practitioner assists in filing the application.
C. Homeless Prevention and Rapid Re-housing program. Funded under the federal stimulus project, this program assists persons who are at risk of being homeless or are homeless in obtaining housing and income.

IV: Behavioral Health –

A. Chemical Dependency Clinic – A Minnesota Rule 31 clinic, this program provides integrated treatment for persons with a substance use disorder and those with co-occurring disorders.
B. Out Patient Mental Health Clinic – Diagnostic assessments, psychological testing, individual, family, or group therapy.
C. Dialectical Behavior Therapy (DBT) Clinic – This program provides services to individuals with Borderline Personality Disorder

V: SMHS Training Institute –

A. The training institute provides clinical training to approximately 800 health care professionals and practitioners a year. There are approximately 45 different courses offered covering a number of evidence-based practices.
B. The agency also has a state grant to provide statewide specific training and consultation in the area of Illness Management and Recovery.

Rationale for the DBT Clinic

Individuals with Borderline Personality Disorder and chronic suicidal behavior are quite prevalent. Research indicates that there is a 5.9% lifetime prevalence for this disorder; this is just under ~12 million people. (J Clinical Psychiatry March 11, 2008:e1-e13) People with this disorder account for approximately 20% of Emergency Room admissions (National Education Alliance on Borderline Personality Disorder). Clearly, this disorder requires assistance to those in need in order to decrease the use of emergency services and costs to the health care delivery system.

More importantly, the disorder requires treatment in order to facilitate positive outcomes for people living with BPD. People with BPD that is untreated are living a painful existence. 10 % of adults with BPD commit suicide. 75% will attempt suicide. A person with BPD has a suicide rate 400 times greater than that of the general public. A young woman with BPD has a suicide rate 800 times greater than the general public. 55-85% of people with BPD self-injure their bodies (National Education Alliance on Borderline Personality Disorder). These statistics not only show the great risk for death for people with the disorder, they also indicate the need for use of a treatment that works.

DBT, Dialectical Behavior Therapy, is an Evidence Based Practice that is shown to be effective in treating BPD. The treatment was developed by Marsha Linehan from the University of Washington in Seattle; Linehan continues to engage in research about the treatment at this time. According to www.behavioraletch.org , recipients of DBT vs. treatment as usual are less likely to drop out of treatment, less likely to engage in parasuicide, are likely to have fewer episodes of parasuicidal behavior that are less severe, are less likely to be hospitalized, are likely to have fewer days in the hospital, and are likely to have higher scores for global and social adjustment.

The NREPP, SAMHSA's National Registry of Evidence-based Programs and Practices, follows some additional outcome measures. In studies for DBT utilized with individuals with drug addiction, individuals receiving DBT were more likely to have drug free urine screens four months after treatment ended. Individuals with eating disorders receiving DBT were far less likely to engage in bingeing or purging than those awaiting treatment.

In summary, people with BPD that is untreated are in pain, suffering, and pose significant likelihood of harm to themselves and/or others. The effects of untreated BPD add costs to our already burdened mental health system. DBT, when used adherently, works to improve the quality of life for those who have BPD while also reducing costs associated with inadequate treatment.

Program Mission

The mission of the DBT Clinic is to improve the lives of those with Borderline Personality Disorder by utilizing Dialectical Behavioral Therapy in a manner that is most adherent to the modes and functions delineated in this evidence-based practice.

Referral and Provider Assignment

The DBT Clinic will accept provider referrals as well as self-referrals. When individuals call about the South Metro Human Services DBT program, they will be sent a referral form by email, fax or mail. If it is an internal referral, staff may find an updated referral form in the Credible electronic medical record. In addition to the referral form, the following will be requested prior to consideration for admission: the most recent diagnostic assessment, hospital discharge summary, and/or pre-petition screening report.

Once the referral forms have been received, the DBT Clinic receptionist will determine insurance eligibility. If the client is eligible, the Program Director will review the referral to ensure that the individual meets the admission criteria for the DBT Clinic.
If it is determined that they do meet criteria, the Clinic Receptionist will call to schedule an intake appointment. The client will be instructed to bring a current health insurance card to the appointment. At the intake appointment, the Clinic Receptionist will enter the client into Credible.

Once the intake has been completed, the Team will review the intake materials and determine if the client is appropriate for admission. If the client is determined to be appropriate, the Program Director will assign the client to a skills group and an individual therapist. The assignments will be made based on availability and best fit.

Eligibility Criteria

The following criteria will be used to determine eligibility for admission:

1. Be within 3 months of 18 years of age or older
2. Have mental health needs that cannot be met with other available community-based services or that must be provided concurrently with other community based services.
3. A diagnosis of Borderline Personality Disorder ***or*** multiple mental health diagnoses with behaviors characterized by impulsivity, intentional self-harm and significant risk of death, morbidity, disability and/or severe dysfunction across multiple domains as evidenced by completion of a functional assessment,
4. Understands and is cognitively capable of participating in DBT as an intensive therapy program and is able and willing to follow program policies and rules assuring safety of self and others,
5. Be at significant risk of one or more of the following if DBT is not provided:
    - Mental health crisis
    - Requiring a more restrictive setting such as hospitalization
    - Decompensation
    - Engaging in intentional self-harm behavior manifested by self-injurious behaviors, chronic suicidal thoughts, significant emotional dysregulation, or problems with impulse control (e.g. promiscuity, shoplifting, substance abuse/dependence, assaultive/threatening behavior towards others, eating issues) that are not alleviated with the current treatment regimen,
6. Is not experiencing active psychosis.

Exceptions to the above criteria may be made with the approval of the Team and the Program Director.

Modes of Treatment

Individual Therapy

Individual DBT combines individualized rehabilitative and psychotherapeutic interventions to treat suicidal and other dysfunctional behaviors and reinforce the use of adaptive skillful behaviors. The therapist will 1) indentify, prioritize, and sequence behavioral targets, 2) treat behavioral targets, 3) generalize DBT skills to the client's natural environment through telephone coaching outside of the treatment session, 4) measure the client's progress toward DBT targets, help the client manage crisis and life-threatening behaviors, and 5) help the client learn and apply effective behavior when working with other treatment providers.

Individual therapy sessions will typically be held once a week for 50-60 minutes. The frequency and length may be adjusted if clinically appropriate and if authorized by the insurance provider. For example, it may be clinically appropriate to have sessions for 90 minutes with some clients and twice a week with some clients. This will be reviewed with the Team to ensure that it is clinically appropriate with the exception of times when the therapist decides in-session to lengthen the session. In this case, the therapist must ensure that the decision is being made with regard to the tasks at hand rather than the desire of either the therapist or the client. The individual therapist is the leader of the client's treatment team. All other modes of therapy revolve around the individual therapy. The individual therapist will utilize the DBT treatment strategies that are outlined in the treatment manual, including validation, problem solving, dialectics, change procedures, communication strategies.

The therapist will develop a diary card with the client upon beginning treatment. The client will be expected to complete the diary card daily and bring it to both individual therapy and skills group. If this is not done, the therapist will work with the client around contingencies for the therapy-interfering behavior. The therapist will use the diary card to help guide the session and to adhere to hierarchies in the treatment. The therapist will also utilize behavioral chain analyses for target behaviors. This may be completed verbally or in writing depending on a number of factors. If it is in writing, the chain will scanned into the client's chart and attached to the case note for the session.

If a client misses four groups in a row, he/she will be discharged from the program. All efforts will be made with the client and the Team to prevent this outcome. See Termination and Transfer procedures for further discussion on this process.

Group Skills Training

Group DBT skills training combines individualized psychotherapeutic and psychiatric rehabilitative interventions conducted in a group format to reduce the client's suicidal and other dysfunctional coping behaviors and restore function by teaching the client adaptive

skills in the following areas: 1) cognitive restructuring, anger management, and crisis-management skills necessary tolerate distress and regulate emotion, 2) communication, behavior management, engagement, leisure, and social skills necessary to function in the community and 3) assertiveness, interpersonal, and problem-solving skills necessary for interpersonal effectiveness.

Stage One therapy requires participation in a skills group. This mode of treatment consists of a weekly group conducted in a psycho-educational format. Groups meet for 2-2 ½ hours each week, with up to eight members per group. Clients may also opt to receive skills training individually. This will be reviewed with the team. The skills modules taught are Core Mindfulness, Interpersonal effectiveness, Emotion Regulation and Distress Tolerance. The Core Mindfulness module is taught at the beginning of each of the other three teaching modules. Each group will have a Leader and a Co-leader leading the group. At least one of the two must be a Qualified Mental Health Professional.

The format of the group is as follows. Each group begins with a mindfulness exercise led by the group Leader. This will follow with discussion about each client's mindfulness practice in that instance. After that is complete, the remainder of the first half of group will be homework review. This will include both a reporting of how skills practice went during the previous week as well as a review of the formal homework assignments from the previous week. In addition, each client is expected to set a goal for the week and report on its completion during this time. If the client is struggling with completion of the goal, the group leaders will coach the client on problem solving strategies to accomplish the goal. If the homework and/or diary card is not complete, a brief chain analysis will be completed with the client to determine the obstacles and problem solve with the goal of helping the client complete homework and practice the skills. Following a break, the new skills will be taught in a didactic format with participation, discussion and questions involving the clients. Clients will be asked to do an observe and describe exercise at the beginning and at the end of each group to assist checking in about their current mental and emotional state. There will be no discussion of individual treatment targets in skills groups.

The groups will follow a schedule that will allow for six months to go through the entire set of modules once (Core Mindfulness-2 weeks, Distress Tolerance-6 weeks, Emotion Regulation-8 weeks, Interpersonal Effectiveness-6 weeks). Clients will go through the series at least twice during treatment. If a client misses four groups in a row, he/she will be discharged from the program. See Termination and Transfer procedures for further discussion on this process.

<u>Skills Coaching</u>

Clients are able to access their individual therapist between individual therapy sessions. The purpose of these calls is to learn adaptive ways to ask for help and enhance skills generalization between groups and sessions. In addition, phone availability offers an avenue for clients to repair relationships following conflict or misunderstandings without having to wait until the following session.

The skills coaching is provided by the individual therapist for that client. Exceptions to this are therapist vacations and other breaks to prevent burnout. In these cases, skills coaching will be provided by another member of the team. The therapist will have oriented the client, during orientation, to the guidelines of coaching calls and to the therapist's personal limits around phone calls. These may need to be reviewed periodically during treatment, especially at the beginning. If the client has a difficult time making the calls, the therapist may set up coaching call appointments to expose the client to calling. Calls will generally no more than 10-15 minutes and will be focused on helping the client get through the immediate crisis without engaging in target behaviors. Further problem solving will be addressed during the next individual therapy session. These calls are not to replace individual therapy sessions nor are they to engage in individual therapy on the phone.

Consultation Team Meetings

The purpose of the consultation team is to help clinicians identify and address problems arising in the delivery of their treatment. It serves as a forum to ensure clinicians do not fall into iatrogenic behaviors, and if they do, that these issues are addressed quickly. The focus is on treating the therapist, not case consultation, using DBT strategies.

The Consultation Team meets weekly for 1 ½ hours. It is attended by skills trainers, individual therapists and the Clinical Leads of the DBT-based Adult Foster Care houses that are also located within South Metro. It is an expectation of each of these individuals that they attend each week with the exception of vacations. Absences other than these are explained to the team by the individual and for any absence a relationship repair is made. The team follows an agenda (see appendix) and there is an assigned Leader, Observer and Note-taker. Each has prescribed responsibilities during the meeting. These roles rotate. Each member of the group signs up to do these roles for certain weeks. Minutes of the meeting are kept and typed by the Note-taker. These are placed in a password protected folder on the agency computer network with access restricted to members of the DBT Team.

Ancillary Treatments

Clients may be involved in treatments in addition to DBT. The individual therapist, with consultation with the Team, will determine if ancillary treatments are appropriate and indicated. Examples of these ancillary treatments are: pharmacotherapy, day treatment, employment programming, 12 step programs, case management, ARMHS, IRTS, Adult Foster Care, specialized eating disorder treatment or other outside groups. With these ancillary treatments, the consultant-to-the-client approach will usually be employed. The individual therapist will coach the client on interacting and communicating with other members of the team. The individual therapist will communicate with non-DBT providers in the following circumstances: if the provider agrees to work with the client based on DBT principles and essentially becomes part of the team, with the client's permission, in an emergency, when it is humane to do so and when the environment is more powerful than the client. The therapist will attend meetings regarding the client's treatment only with the client's permission and when the client will also be present. The

focus will be on coaching the client to speak for and advocate for him/herself. The client will be encouraged and coached to set up treatment meetings for him/herself.

Stages of Treatment

Pre-Treatment

This phase of treatment should last 3-4 sessions. There may be circumstances where this will take longer and will be reviewed with the Team and Program Director. The first meeting will be an intake appointment with the Program Director or designee. In this meeting, the client will complete the Borderline Symptom List-23. The intake will be a screening to determine if the client meets admission citeria and is willing to commit to the treatment. If the client meets criteria and agrees to admission, they will be assigned to an individual therapist. The therapist will complete either a Standard or Extended Diagnostic Assessment, depending on need. They will also complete a functional assessment, gather prior treatment history (e.g. repeated hospital admissions, ER events, crisis services and use of other intensive human services as well as experiences in psychotherapy) and completion of the Borderline Symptom List-23. These tools will be used to determine the client's eligibility for admission. A diagnostic assessment will be completed and updated every year thereafter. After three years, a full diagnostic assessment will be completed.

Orientation and Commitment

The next 2-3 sessions will encompass orientation and commitment to the DBT treatment and program. The client will be informed that these sessions will be used for orientation and to give both parties the opportunity to decide if they can work together as well as if the therapist can treat the presenting problems. Continuing therapy will not be ensured until after obtaining the requisite commitments discussed below. During the first sessions, the assigned therapist and the client will work on identifying life goals and will discuss how DBT can help the client achieve them. The therapist will cover the items in the orientation checklist (in no particular order) which will outline everything to be discussed in the first few sessions including commitment. In addition, the therapist will include an explanation of findings from the assessment/evaluation process. This will be documented in an orientation and commitment note in the client's chart. In the first session with the therapist the client will be given a binder that includes the handouts for skills groups and material describing the program. In addition, the binder will contain handouts of the orientation topics. The client will be given the therapist's phone number for skills coaching as well as other emergency numbers. The client will be oriented to the purpose of the calls as well as the therapist's personal limits around calls. Orientation will also include the biosocial theory; the structure of DBT; the expectations and roles of the client (including contingencies for missing sessions), skills trainers and therapists; diary cards; behavioral chain analyses; and contingencies. In addition, the client will be instructed on how to orient his/her family, friends and providers to the treatment. If possible and appropriate, a joint meeting will be set up with the therapist, client and a family member, friend and/or provider in order to foster communication about current risk and to discuss the consultant-to-the-client role that the therapist will be playing within the network.

The client and therapist will discuss the expectations of the therapy, therapist and the client. If life-threatening behaviors (including non-suicidal self-injury) are present, the client will

be advised that reducing these behaviors is a primary treatment goal and they must commit to working toward solving problems in ways that do not include intentional self-harm or suicide. The therapist will conduct chain analyses of major life threatening behaviors and discuss the client's previous experiences in therapy during this period. After reviewing all of the information, the client will be asked to make a commitment to the treatment with all of the outlined agreements. A treatment authorization will be requested from the client's insurance provider. When the authorization is received and after the client and the therapist agree to work together, they will both sign a contract committing to completing one year of DBT treatment and to all the expectations discussed in the first few sessions. This will be located in the client's Credible chart and the client will be given a copy. At this time, a treatment plan will be completed with the client and reviewed every 90 days thereafter.

Stage One Treatment

Stage One treatment will focus on reducing suicidal behaviors, therapy-interfering behaviors, major quality-of-life-interfering behaviors, and increasing behavioral skills. The focus will be on obtaining a reasonable life expectancy and behavioral/emotional regulation. Stage One treatment will include weekly individual therapy sessions and weekly skills group attendance as well as telephone skills coaching to integrate the skills into the client's life. This stage will continue for at least one year. At approximately 10-11 months, the therapist, client and team will evaluate the client's progress. If the following criteria are met, there may be a decision to extend treatment.

Continuing Stay Criteria

1. The client is actively participating and engaged in the DBT Program, its treatment components and guidelines in accordance with the treatment team expectations.
2. There is demonstrable progress as measured against the client's baseline level of functioning prior to the DBT intervention. Examples of demonstrable progress may include:
    - Decreased self-destructive behaviors
    - Decrease in acute psychiatric symptoms with increased functioning in activities of daily living
    - Showing objective signs of increased engagement
    - Reduction in the number of acute care services, e.g. ED visits, crisis services, hospital admissions
    - Applying skills learned in DBT in crisis situations
3. Although the pace of progress may vary, the client continues to make progress toward goals but has not fully demonstrated an internalized ability to self-manage and use learned skills effectively.
4. The client is actively working towards discharge, including concrete planning for transition and discharge.
5. The continued need for treatment as indicated in the above criterion has ongoing documented evidence in the client record.

If these criteria are met and the client chooses to continue treatment, a six month authorization will be requested from the insurance provider for continuing treatment. The client's progress will be reviewed every six months thereafter until it is determined

by the team and the client they meet criteria for discharge as outlined below. At the 12 month and six month reviews, discharge determinations will follow the following criteria.

Discharge Criteria

1. The client's individual treatment plan goals and objectives have been met or the client no longer meets the continuing stay criteria as outlined above.
2. The client's thought, mood, behaviors or perception have improved to a level that a lesser level of service is indicated.
3. The client chooses to discontinue the treatment contract.
4. Ongoing clinical assessment leads to the conclusion that the client no longer meets admission criteria or another treatment modality would be more efficacious.
5. Clinically determined discharge (see termination and transfer procedures below).

If the client meets criteria for discharge from the comprehensive DBT program, then a determination will be made whether to refer to Stage Two treatment. A client may choose to discharge at the end of Stage One, continue to Stage Two treatment with their current therapist or be referred elsewhere for Stage Two treatment.

Stage Two Treatment

If a client chooses to continue into Stage Two treatment, a discussion will take place about the goals and needs of the client and if they can be met with their current therapist. If the therapist and client agree that continuing the relationship is comfortable and advantageous to the client, treatment may continue without a referral. This will be done with the approval of the Program Director. A referral may be made to another therapist or agency if either the client or the therapist are uncomfortable with the relationship continuing beyond the agreed upon period. The option to continue with Stage Two treatment within the agency may be available with another therapist on the Team. If not or if the client prefers, a referral will be made to therapists or agencies that have the competencies to provide the treatment. The client will be provided with names and it will be their responsibility to determine which provider they will use.

If the decision is to proceed with Stage Two therapy within the agency, the therapist and the client will discuss goals of treatment. The focus is on working directly on posttraumatic stress as well as continued progress toward the client's individual goals and improving self-respect and functioning within life.

Termination and Transfer Procedures

When termination occurs at the end of a treatment contract, the therapy sessions will taper rather than go from weekly to no therapy. The therapist will discuss with the client progress that has been made, express confidence in the client's abilities to be outside of therapy, emphasize that caring and concern for the client do not end and discuss the natural and community resources the client has to call upon. If a referral is necessary to another provider or program, the therapist will facilitate the referral. (See explanations of Stage One and Stage Two treatments above.)

Termination prior to the end of the contract will only be done after the therapist has worked very clearly and with close attention to therapy interfering behaviors and sought consultation and used the strategy of therapy vacations. Vacations from therapy may be employed as a contingency to assist the client in making necessary behavior change when there are situations or behaviors that severely interfere with therapy. When a therapy vacation occurs, the client will be warned about the possibility of the vacation and be informed of what will need to happen to prevent it. They will then be given the opportunity to prevent the vacation. If a client goes on vacation, the therapist will maintain some minimal contact to encourage the client to do what is necessary to be able to return to therapy.

If a client misses four group or four individual sessions consecutively, the client will be discharged from the program. The expectation of the individual therapist is to identify with the client the therapy-destroying behavior prior to the fourth missed session in order to obtain a re-commitment and get the client to return to treatment. The therapist will be expected to discuss the situation with the Team. If the client is discharged for missing sessions and they wish to return, they will need to have a conversation with their individual therapist to identify why they would like to return and why their participation will be different during the new admission. The Team will review the request and determine the appropriateness of the re-admission and what contingencies may need to be in place.

There may be some cases under which it would be advisable to transfer the client's therapy to another therapist. Mostly, this would happen near the beginning of treatment if the therapist believes that anther therapist would be more helpful if there is another therapist available. This will be done in consultation with the team and supervision with the Program Director.

Staffing requirements:

DHS will be notified by email (dbt.certification@state.mn.us) of any changes in team composition within 10 days, including hiring of new staff, staff terminations and when a staff member obtains an NPI.

Team Leader (Program Director): The DBT Team Leader will meet the following qualifications: 1) be a mental health professional employed by, affiliated with, or contracted by a DBT program certified by the commissioner, 2) have appropriate competencies and working knowledge of the DBT principles and practices, and 3) have knowledge of and ability to apply the principles and DBT practices that are consistent with evidence-based practices. The Team Leader must be enrolled as a provider with Minnesota Health Care Plans (MHCP) and have completed and submitted form DHS-6330-ENG, "Qualified Mental Health Professional Clinical Supervision Assurance Statement" if there are Clinical Trainees as part of the team.

Team members who provide individual DBT or group skills training must: 1) be a Mental Health Professional or Mental Health Practitioner qualified as a Clinical Trainee employed by, affiliated with, or contracted by a DBT program certified by the

commissioner, 2) have or obtain competencies and working knowledge of DBT principles and practices within the first six months of becoming apart of the DBT Program, 3) have or obtain competencies and ability to apply the principles and practices of DBT consistently with evidence-based practices within the first six months of working at the DBT program, 4) participate in DBT consultation team meetings, and 5) Mental Health Practitioners are required to have ongoing clinical supervision by a Mental Health Professional who has appropriate competencies and working knowledge of DBT principles and practices. All Mental Health Professionals must be enrolled as providers in Minnesota Health Care Plans (MHCP).

Orientation and Commitment of New Staff

New members to the team will meet with the Team Leader prior to attending sessions. In this meeting will be a presentation of the purpose of consultation team and explanation of the Linehan model of consultation team; roles, responsibilities and purpose of Leader, Observer and Note-taker; orientation to mindfulness; a review of group norms and assumptions of clients and therapy; the use of chain analyses, secondary targets and the didactic; consultation agreements and commitments. The clinician will need to commit to the team and sign that they have received orientation and agree to commit to the team with all the expectations and responsibilities.

Ongoing Training Expectations

All team members will complete the continuing education requirements of their licenses and/or requirements of South Metro Human Services. During the consultation team meeting will be an educational presentation once a month. All members will be expected to do an additional 10 hours of continuing education directly related to DBT-suicide risk assessment, biosocial theory and framework of DBT, validation, mindfulness, skills training, consultation, dialectics, etc. Advanced training on exposure based procedures, cognitive modification, contingency management and behavioral analysis will also be required. Continuing education can be completed in-person, on DVD or online. Each Team member will be required to track training and provide documentation to the Program Director.

Quality Monitoring/Improvement Processes

The DBT Clinic will comply with DHS mandated data collection. This will include completion of the the Program Outcomes Status Report (POSR) and the DBT Supplemental Information to the POSR as well as the Borderline Symptom List-23 (BSL-23). These (with the exception of the BSL-23) will be completed at admission, every six months therafter and at discharge. The data will be sent to DHS with other agency POSR data at specified dates twice a year. The BSL-23 will be completed at admission and at every module change in the skills groups as well as at discharge. These will be monitored for changes in symptom level and maintained in each client's chart. In addition, a skills trainer evaluation will be completed by the clients at 6 month intervals.

Orientation and Commitment Note

This individual is completing the orientation and commitment process for admission to the DBT Clinic. The following items were completed and/or strategies used in this session.

(check boxes)

- Completed diagnostic interview
  [embed DA for completion]
- Orient client to purpose of initial sessions
- Give client treatment binder and list of DBT team
- Identify current problems the client wants help with and their life goals and explain concept of building a "Life Worth Living"
  (text box)
- Identify how DBT can help client achieve life goals
- Review biosocial theory
- Orient to dialectics
- Explain diagnosis, especially using Emotional, Interpersonal, Self, Cognitive and Behavioral Dysregulations
- Review structure of DBT, including the target hierarchy and structure of group
- Review expectations and roles of client, skills trainers, therapist and consultation team, including the role of collaboration
- Review purpose of phone coaching and limits of individual therapist
- Give phone number for phone coaching
- Orient to 24 hour rule
- Discuss contingencies for missed sessions/groups
- Orient to use of diary card
- Orient to use of behavioral chain analyses
- Discuss problem solving model of behaviors such as intentional self-injury and rationale for skills as alternative solutions
- Discuss video taping sessions and obtain release if willing
- Discuss why Stage 2 treatment is contraindicated in Stage One treatment
- Instruct client on orienting family, friends, providers to DBT
- If appropriate, set up meeting with family, friends or providers to foster communication about risk and to discuss the consultant-to-the-client role that the therapist will be playing within the network
  (text box-prompt Document meeting)
- Inform client that reducing life threatening behaviors is a primary treatment goal and they must commit to working toward solving problems in ways that do not include intentional self-harm or suicide. Obtain commitment.
  (text box-prompt Document)
- Utilize commitment strategies to obtain necessary commitments: (check boxes)
- Sell commitment: evaluate the pros and cons
- Play the Devil's Advocate
- Use foot-in-the-door and door-in-the-face
- Connect present commitment to prior commitments
- Highlight the freedom to choose and the absence of alternatives

- Shaping
- Conduct chain analysis of major life threatening behavior
  (text box for explanation) [Prompt: Attach copies of chains to the note.]
- Review previous experiences in therapy
  (Text box for explanation)
- Obtain commitment from client and sign contract for one year of therapy
  (Embed treatment contract)
- Other (text box)

(Text box for any explanation)

Tx DBT Txmt Plan

*Please confirm the ACCURACY of the START AND REVIEW DATES .

Start Date:*

Review Date:*

*ADMINISTRATIVE USE ONLY. PLEASE DO NOT ENTER AN END DATE.

End Date:

*The following are guidelines in developing the DBT treatment plan goals, objectives, interventions. The goals, objectives and interventions must be detailed and specific; avoiding generalities.

*Goals: What are the things the client wants to accomplish in her/his life? *Objectives: What are the treatment targets? What behaviors do we want to decrease and what behaviors do we want to increase?

1: Life Goal:

Objectives:

Interventions (check those that apply):
☐ 50-60 minutes weekly of Individual Therapy
☐ 50-60 minutes twice weekly of Individual Therapy
☐ 75-90 minutes weekly of Individual Therapy
☐ 75-90 twice weekly of Individual Therapy
☐ DBT skills training group - 120 minutes weekly
☐ Skills coaching - 5-15 minutes as needed

2: Life Goal:

Objectives:

Interventions (check those that apply):
☐ 50-60 minutes weekly of Individual Therapy
☐ 50-60 minutes twice weekly of Individual Therapy
☐ 75-90 minutes weekly of Individual Therapy
☐ 75-90 twice weekly of Individual Therapy
☐ DBT skills training group - 120 minutes weekly
☐ Skills coaching - 5-15 minutes as needed

3: Life Goal:

Objectives:

Interventions (check those that apply):
☐ 50-60 minutes weekly of Individual Therapy
☐ 50-60 minutes twice weekly of Individual Therapy
☐ 75-90 minutes weekly of Individual Therapy
☐ 75-90 twice weekly of Individual Therapy
☐ DBT skills training group - 120 minutes weekly
☐ Skills coaching - 5-15 minutes as needed

_____
Staff Name:
Title:

Individual Therapy Prog Note

Mental Status Affect:

- ☐ Broad
- ☐ Blunted
- ☐ Labile
- ☐ Inappropriate
- ☐ Other _____

Mood:

- ☐ Dysthymic
- ☐ Dysphoric
- ☐ Euthymic
- ☐ Elevated
- ☐ Euphoric
- ☐ Other _____

Behaviors Addressed (check all that apply):

- ☐ Life threatening
- ☐ Therapy interfering
- ☐ Quality of life interfering
- ☐ Unrelenting crisis
- ☐ Inhibited emotion experiencing
- ☐ Active passivity
- ☐ Apparent competence
- ☐ Emotion vulnerability
- ☐ Self invalidation

Diary card completed?

- ☐ Yes

If Yes, please attach the diary card to the progress note.

- ☐ No

If NO, was diary card completed in session? If no, reason:

_____

Self harm?

- ☐ Yes

*[Written chain should be attached to the progress note.]
*[Verbal chain should be clearly documented.]

- ☐ No

If No, reason: _____

Suicidal ideation?

- ☐ Yes

If Yes, consider completing suicide risk assessment, especially if level is outside the norm.

☐ No

Suicidal plan?

☐ Yes

If Yes, must complete suicide risk assessment.
Upon completion of suicide risk assessment, outline outcome and plan.

☐ No

Life Goals/Objectives

Life Goal 1:

Objective 1 :

Describe the Intervention(s)

Interventions:

☐ Diary card review
☐ Formulate treatment hierarchy
☐ Behavior chain analysis
☐ Solution analysis
☐ Skills training
☐ Contingency management
☐ Exposure
☐ Cognitive restructuring
☐ Self management
☐ Orientation
☐ Validation
☐ Dialectical strategies
☐ Behavioral rehearsal
☐ Case management strategies
☐ Treatment planning
☐ Crisis interventions
☐ Commitment strategies
☐ Didactic strategies
☐ Reciprocal communication
☐ Irreverent communication
☐ Insight interpretation

Result of the contact:
*[Identify individual response to interventions, progress made toward goal/objective or resolution of crisis situation.]

Plan for next contact:
*[Who will do what? Health and safety issues and plan to address them? Homework assigned.]

Life Goal 2:

Objective 2 :

Describe the Intervention(s)

Interventions:

- ☐ Diary card review
- ☐ Formulate treatment hierarchy
- ☐ Behavior chain analysis
- ☐ Solution analysis
- ☐ Skills training
- ☐ Contingency management
- ☐ Exposure
- ☐ Cognitive restructuring
- ☐ Self management
- ☐ Orientation
- ☐ Validation
- ☐ Dialectical strategies
- ☐ Behavioral rehearsal
- ☐ Case management strategies
- ☐ Treatment planning
- ☐ Crisis interventions
- ☐ Commitment strategies
- ☐ Didactic strategies
- ☐ Reciprocal communication
- ☐ Irreverent communication
- ☐ Insight interpretation

Result of the contact:
*[Identify individual response to interventions, progress made toward goal/objective or resolution of crisis situation.]

Plan for next contact:
*[Who will do what? Health and safety issues and plan to address them? Homework assigned.]

Life Goal 3:

Objective 3:

Describe the Intervention(s)

Interventions:

- ☐ Diary card review
- ☐ Formulate treatment hierarchy
- ☐ Behavior chain analysis
- ☐ Solution analysis
- ☐ Skills training
- ☐ Contingency management
- ☐ Exposure
- ☐ Cognitive restructuring
- ☐ Self management
- ☐ Orientation
- ☐ Validation
- ☐ Dialectical strategies
- ☐ Behavioral rehearsal
- ☐ Case management strategies
- ☐ Treatment planning
- ☐ Crisis interventions
- ☐ Commitment strategies
- ☐ Didactic strategies
- ☐ Reciprocal communication
- ☐ Irreverent communication
- ☐ Insight interpretation

Result of the contact:
*[Identify individual response to interventions, progress made toward goal/objective or resolution of crisis situation.]

Plan for next contact:
*[Who will do what? Health and safety issues and plan to address them? Homework assigned.]

Date: _____

_____
Staff Name:
Title:

DBT Skills Training Note

Session focused on:

- ☐ Core Mindfulness
- ☐ Distress Tolerance
- ☐ Emotion Regulation
- ☐ Interpersonal Effectiveness
- ☐ Other _____

Homework assigned:

Participated in mindfulness?

- ☐ Yes
- ☐ No

Observations: _____

Homework completed?

- ☐ Yes
- ☐ No

Comments:_____

Diary card completed?

- ☐ Yes
- ☐ No

Comments: _____

Completed goal from last session?

- ☐ Yes
- ☐ No
- ☐ Partial

Comments: _____

New goal?

Demonstrated understanding of the topic:

- ☐ Poor
- ☐ Fair
- ☐ Good
- ☐ Excellent

Comments: _____

Showed listening skills/empathy/respect:

- ☐ Poor
- ☐ Fair
- ☐ Good
- ☐ Excellent

Comments: _____

Offered opinions/suggestions/feedback:

- ☐ Poor
- ☐ Fair
- ☐ Good
- ☐ Excellent

Comments: _____

Date: _____

_____

Staff Name:
Title:

***ASSESS: Crisis Plan

Prior to cloning this document, confirm that the client's diagnosis, address, phone number etc. has not changed since last Crisis Plan was created. A new Crisis Plan must be created if this has changed.

Crisis Plan: ---SELECT---
Effective Date:
Client's Phone Number:
Client's Alternative Phone Number:
Client's Address Notes:
Date of Birth:

Physical Description:

Race:*
Height: (ft. in.)*
Weight: (lbs.)*
Eye Color:*
Hair Color:*
Identifying Marks: (Scars, tattoos, etc.)
Other critical information: (eg. vehicle description, driver's license # etc.)

+

Insurance Carrier:
Client's PMI#
Hospital Preference:
---SELECT---
MA Restriction

- No
- Yes

Contacts:

- Emergency Contact

- Alternative Decision Maker

- Psychiatrist Name

- Primary Care Physician

- Community Treatment Provider(s)/Case Manager

- Rep Payee

- CADI

- Therapist

- Probation/Parole Officer

Other Contacts (**Include Contact Type, Name, Phone Number and any additional information needed.)

Legal Status:

Under Commitment

- No
- Yes

Other Legal Concerns (please include hx of/current criminal concerns)

+

Advance Psychiatric Directive on file

---SELECT---

Diagnosis:

Medical or Other Health Concerns

+

History of dangerous behavior to self or others (Include past, recent and current history)

+

Risk factors and potential crisis situations

Medication Non-Compliance?

Yes

No

Drugs/Alcohol

Yes

No

Weapons in the household

Yes

No

Violence/aggressiveness individuals, drug activity, aggressive animals

Yes

No

Other risk factors

Yes

No

Appropriate crisis interventions:

+

Complete

* *Indicates required field*

IMMINENT SUICIDE RISK AND TREATMENT ACTIONS NOTE

Client Name: _____ Contract Date: _____

Therapist Name:_____ Today's Date:_____

REASON FOR IMMINENT RISK AND TREATMENT ACTION NOTE

1)

☐ HISTORY of suicide ideations, suicide attempts, or intentional self-injury at intake (check only 1st session)
☐ NEW (or first report of) suicide ideation ☐ fleeting ☐ frequent ☐ continual
☐ INCREASED suicide ideation
(Describe):
_____
_____
☐ THREAT or other behavior indicating IMMINENT SUICDE RISK
☐ ATTEMPT since last contact
☐ CURRENT suicide attempt
(Describe):_____
_____

2) **IMMINENT suicide risk factors**

Not Reported Not Observed	NO	YES	SUICIDE RISK FACTORS	COMMENT
☐	☐	☐	HISTORY of suicide attempts/self-injury	
☐	☐	☐	CURRENT suicide intent, including client belief that he/she is going to commit suicide or hurt self	
☐	☐	☐	Preferred METHOD CURRENTLY or easily AVAILABLE	
☐	☐	☐	LETHAL MEANS (of any sort) CURRENTLY or easily available	
☐	☐	☐	CURRENT PLAN and/or preparation (including specific method and time)	
☐	☐	☐	CURRENT PRECAUTIONS against discovery; deception about timing, place,	

			etc.	
☐	☐	☐	CURRENT SUBSTANCE USE, including ETOH and Rx meds (last 3 hours)	
☐	☐	☐	Currently or will be ISOLATED or ALONE	
☐	☐	☐	PROMPTING EVENTS for previous parasuicide	
☐	☐	☐	SUDDEN LOSS, other negative event	
☐	☐	☐	ABRUPT CLINICAL CHANGE, either negative or positive	
☐	☐	☐	INDIFFERENCES/DISSATISFACTION with therapy	
☐	☐	☐	First night of INCARCERATION	
☐	☐	☐	Current Severe HOPELESSNESS	
☐	☐	☐	Current MAJOR DEPRESSION PLUS:	
☐	☐	☐	Current Severe TURMOIL, ANXIETY, PANIC attacks, mood CYCLING	
☐	☐	☐	Current Severe, GLOBAL INSOMNIA	
☐	☐	☐	Current Severe ANHEDONIA	
☐	☐	☐	Current Inability to CONCENTRATE, INDECISION	
☐	☐	☐	Current PSYCHOSIS, voices telling client to commit suicide	
☐	☐	☐	CHRONIC PHYSICAL pain	
☐	☐	☐	USUALLY OR CURRENTLY HIGHLY IMPULSIVE	
☐	☐	☐	Client MOTIVATED TO UNDER REPORT/LIE about risk	Required if no:
☐	☐	☐	OTHER:	

3) IMMINENT suicide protective factors: (Check ALL)

Not Reported Not Observed	NO	YES	PROTECTIVE FACTOR:	COMMENT
☐	☐	☐	HOPE for the future	
☐	☐	☐	SELF-EFFICACY in problem area	
☐	☐	☐	ATTACHMENT to life	
☐	☐	☐	RESPONSIBILITY to children, family or others, including pets, who client would	

			not abandon	
☐	☐	☐	ATTACHED to therapy and at least one therapist	
☐	☐	☐	THERAPIST attached, will stay in contact	
☐	☐	☐	Embedded in PROTECTIVE SOCIAL NETWORK or family	
☐	☐	☐	FEAR of active suicide, death and dying or no acceptable method available	
☐	☐	☐	Fear of SOCIAL DISAPPROVAL for suicide	
☐	☐	☐	Belief that suicide is IMMORAL or that it will be punished; HIGH spirituality	
☐	☐	☐	COMMITMENT to live and history of taking commitments seriously or reason to trust this commitment	
☐	☐	☐	Client WILLING TO FOLLOW CRISIS PLAN	
☐	☐	☐	Client MOTIVATED TO OVER-REPORT risk	Required if yes:
☐	☐	☐	Other (described):	

4) **Treatment actions aimed at suicidal behaviors (Check All that apply)**

A. ☐ Did CHAIN ANALYSIS of suicidal ideation and behaviors (attach written chain to note or clearly document).

B. ☐ Focused on CRISIS INTERVENTION and/or PROBLEM SOLVING (Check those used):

- ☐ VALIDATED current emotions and wish to escape or die (emotional support).
- ☐ Worked to remove or remediate PROMPTING EVENTS
- ☐ Gave advice and instructed in use of COPING SKILLS to reduce suicidality
  - ☐ Crisis Survival
  - ☐ Mindfulness
  - ☐ Emotion Regulation
  - ☐ Interpersonal Effectiveness
  - ☐ Self-Management
  - ☐ Generate HOPE and reasons for living
  - ☐ OTHER

  COMMENTS on crisis intervention:

C. ☐ Developed or reviewed existing CRISIS PLAN
D. ☐ Committed to a PLAN OF ACTION

☐ Client made credible AGREEMENT for crisis plan and no self-injury or suicide

attempt attempts until (DATE): _____

☐ Client agreed TO REMOVE LETHAL implements (drugs, knife).

When (date):_____

How:_____

E. ☐ Did TROUBLE SHOOTING of factors that might interfere with effective actions:

F. ☐ Increased SOCIAL SUPPORT

- ☐ Plan for client to contact SOCIAL SUPPORT (who): _____
- ☐ ALERTED NETWORK to risk (describe): _____
- ☐ Planned for FOLLOW-UP call for _____

G. ☐ REFERRED:

- ☐ To Primary Therapist:
- ☐ To Clinician-On-Call at
- ☐ To Crisis Line ☐ (Insured Client Had Phone Number)
- ☐ To _____for medication evaluation:
- ☐ OTHER:

H. ☐HOSPITALIZATION CONSIDERED did not recommend because (check all that apply):

- ☐ Client is **NOT IMMENITIENTLY DANGEROUS**
- ☐ Other environmental support available
- ☐ Client can easily contact me if condition worsens
- ☐ Client previously hospitalized, benefit not apparent
- ☐ No bed available
- ☐ Client refused
- ☐ Client refused even with persistent argument with me in favor
- ☐ Client does not meet criteria for transport hold

and/or it would (check all that apply):

- ☐ Increased stigma and isolation which are important issues for this client
- ☐ Interfere with work or school which are important for this client
- ☐ Violate already agreed to plan
- ☐ Cause undue financial burden which is an important issue for this client

☐ OTHER: describe

5) **I believe, based on information currently available to me (check all that apply)**

A. ☐ Client is **NOT <u>IMMINENTLY</u> DANGEROUS** to self and will be safe from serious self-injury or suicide until next contact with me or with primary therapist <u>for the following reasons</u>: (Check all that apply)

- ☐ Problems that contribute to suicide risk are being resolved
- ☐ Suicide ideation and/or intent reduced by end of contact
- ☐ Credible agreement for crisis plan and no self-injury or suicide attempts
- ☐ Adequate crisis plan in place
- ☐ Suicidality being actively addressed by primary therapist
- ☐ Protective factors outweigh risk factors

☐ Other:

B. ☐ There is some **IMMINENT DANGER** of serious self-injury or suicide. However, emergency interventions likely to exacerbate rather than resolve long term risk.

C. ☐ Emergency intervention is needed to prevent **IMMINENT DANGER** of medically serious self-injury or suicide (Check all that apply).

☐ Took to ER at _____
☐ Arranged for outreach evaluation for Transport Hold
(Describe):_____
☐ Completed transport hold and arranged transport to hospital
with_____
☐ Arranged for a **POLICE WELLNESS CHECK**
☐ **CALLED 911** for medical aid.
☐ HOSPITALIZATOIN ARRANGED at: , to be admitted by Dr.:
on (day)
COMMENTS on emergency intervention

D. ☐ Significant **UNCERTAINTY EXISTS** as to imminent risk, I will get a second opinion from: (check all that apply).

☐ SUPERVISOR: _____ ☐ PSYCHIATRIST:_____
☐ TEAM MEMBER or COLLEAGUE_____ ☐ PRIMARY THERAPIST _____
☐ OTHER:
_____

6) **Client will be REEVALUATED for suicide risk no later than (Default= next session):**

1) ☐ 12 hours, how?_____
2) ☐ 24 hours, how?_____
3) ☐ 48-72 hours, how?_____
4) ☐ Next individual session
5) ☐ Next group session
6) ☐ Next pharmacotherapy session
7) ☐ OTHER: Describe:_____

Diary Card:
Goals:

Day/Date	SIB urges (0-10) Action Y or N	Suicidal ideation (0-10)/ Planning Y or N	Purging Urge (0-10) Action Y or N (#)	Restriction (0-10) Action Y or N	Hopeless-ness (0-10)	Fear (0-10)	Joy (0-10)	Night-mares Flashbacks	Lost time? Y/N	Being dishonest (0-10) Action Y or N	Meds as prescribed Y/N
Monday											
Tuesday											
Wednesday											
Thursday											
Friday											
Saturday											
Sunday											

DBT Skills Diary Card	Filled out this side? ___ Daily ____ 2-3x ____ 4-6x ____Once ____ In session	**Check days skill was practiced on purpose**						
		MON	**TUE**	**WED**	**THUR**	**FRI**	**SAT**	**SUN**
1. Wise mind								
2. Observe: *just notice*								
3. Describe: *put words on, just the facts*								
4. Participate: *enter into the experience*								
5. Non-judgmental stance								
6. One-mindfully: *present moment*								
7. Effectiveness: *focus on what works*								
8. Figure out interpersonal goals and priorities								
9. DEAR *Describe, Express, Assert, Reinforce*								
10. MAN *Mindful: Broken record, Ignore attacks, Appear confident, Negotiate*								
11. GIVE *Gentle, Interested, Validate, Easy manner*								
12. FAST *Fair, no-Apologies, Stick to values, Truthful*								
13. Attend to relationships with VALIDATION								
14. Evaluate Options *for how strongly to ask or say no*								
15. Opposite-to-emotion action								
16. Problem solving								
17. ABC: *Accumulate positives, Build mastery, Cope ahead*								
18. PLEASE: *Treat PhysicaL ills, Balanced Eating, Avoid drugs, Sleep, Exercise*								
19. Mindfulness of Current Emotion								
20. Pros and Cons								
21. Self-Soothe,								
22. Improve the moment								
23. Distract								
24. Radical acceptance *and Turning the Mind*								
25. Willingness								
26. Half-smiling								

DBT Consultation Agenda

Remind team to turn off pagers and cell phones.

Date:
Leader:
Observer:
Note Taker:
Present:
Absent:

11:30am-12pm

1. Mindfulness

2. Agreements/Assumptions

3. Set the agenda:

A. Team Check in:

- **Review last meeting's notes and any case updates.**
- Chain Analysis **:**
- Relationship Repairs:
- Burnout check-in/Diary Cards:

B. 12:00pm Teaching/Consultation (45 minutes):

▪ Client successes:
▪ Therapy destroying behavior:
▪ Life threatening behavior:
▪ Up to three cases for consultation with rating (A, B, C) or complete didactic last Wed of month:
a. b. c.

C. 12:45pm: Administrative Issues

- Announcements:
- Vacations/absences:

D. 12:55pm: Observer Comments:

CONSULTATION TEAM MEETING CHECKLIST

Part A

STRUCTURE OF THE CONSULTATION TEAM MEETING

The Team designated:

- ☐ A Team Leader (TL): _____
- ☐ An Observer (Ob): _____
- ☐ A Note- Taker (NT): _____

☐ The TL led a mindfulness practice.

☐ The TL read one of the Consultation Team Agreements.

☐ The TL read the notes from last week's Team.

☐ The TL checked if anyone was going out of town.

☐ A clinician expressed plan to go out of town: _____

- ☐ The Team identified a back-up/coverage plan for any team member going out of town.

The TL checked if anyone had clients with:

- ☐ Life-Threatening Behavior (including imminent suicide risk):_____
- ☐ Therapy-Destroying Behavior (including approaching 4 misses):_____

☐ The TL checked if any therapists were approaching burnout: _____

☐ At the end of the meeting, the Observer rang the bell to close the meeting.

GENERAL TEAM PROCESS

☐ The team discussions focused primarily on THERAPIST behavior more so than clinical issues with clients.

☐ Generally, the highlighting, targeting and problem solving regarding team members' behaviors were conducted in an "easy manner".

☐ A strong position was expressed about a clinical or related issue.

- ☐ Someone on the team brought up an opposing position.
- ☐ The dialectic or tension was highlighted.
- ☐ The team worked to achieve synthesis.

☐ The team meeting involved a balance of acceptance (validating) and change-based (problem solving) interaction styles.

PART B:

BEHAVIORS DURING THE CONSULTATION TEAM MEETING

☐ A therapist was doing 2 things at once (i.e. reading and listening; talking on the phone; chatting out of turn with other team members)

- ☐ The Observer rang the bell.
- ☐ This behavior was highlighted and blocked by the team.

☐ A therapist was treated as "fragile". An obvious issue came up that needed to be targeted (i.e. defensiveness, judgmental talking, lateness) that was not highlighted or discussed by the team or feedback clearly was needed but was not provided.

- ☐ The Observer rang the bell.
- ☐ This behavior was highlighted.
- ☐ The team discussed the avoided issue or provided the needed feedback.

☐ A therapist displayed defensiveness in response to feedback.

- ☐ The Observer rang the bell.
- ☐ This behavior was highlighted.
- ☐ The therapist was asked to re-phrase the statement(s).

☐ A therapist offered solutions before the problem was clearly defined.

- ☐ The Observer rang the bell.
- ☐ This behavior was highlighted.
- ☐ The problem was clarified.

☐ A therapist engaged in self-invalidation (i.e. denigrating self, judgmental toward self, presenting as "incompetent").

- ☐ The Observer rang the bell.
- ☐ This behavior was highlighted and blocked by the team.
- ☐ The therapist was asked to re-phrase the self-invalidating statement(s).

☐ A therapist spoke in a judgmental or derogatory manner about his or her client(s).

- ☐ The Observer rang the bell.
- ☐ This behavior was highlighted and blocked by the team.
- ☐ The therapist was asked to re-phrase the judgmental statement(s).

☐ A therapist was late for the meeting.

- ☐ The behavior was highlighted.
- ☐ A chain analysis was conducted.
- ☐ Solutions were agreed upon.
- ☐ A commitment to implement a solution was elicited.

☐ A therapist was obviously unprepared.

- ☐ The behavior was highlighted.
- ☐ A chain analysis was conducted.
- ☐ Solutions were agreed upon.
- ☐ A commitment to implement a solution was elicited.

☐ A therapist did not speak during the team meeting.

- ☐ The behavior was highlighted.
- ☐ A brief chain analysis was conducted.
- ☐ Solutions were agreed upon.
- ☐ A commitment to implement a solution was elicited.

DBT Clinic Consultation Team Orientation Checklist

1. _____ Explain reasons for consultation team meeting and its place in the DBT model.
2. _____ Mindfulness
   - Give and review handout on directions for leading mindfulness
   - Orient to location of supplies and options for exercises
3. _____ Roles and responsibilities of Leader
   - Review Agenda sheet
4. _____ Roles and responsibilities of Observer
   - Review Observer Checklist and use of bell
5. _____ Roles and responsibilities of Note-taker
   - Need for notes to be done and why
   - How/where to disperse
6. _____ Review Assumptions and SMHS Consultation Team Agreements.
7. _____ Review Linehan model of consultation.
   - Use handout from ISITDBT
8. _____ Review need for and structure of didactic.
   - Review adherence pyramid
   - Review upcoming certification/credentialing
9. _____ How to ask for and receive consultation
   - Review Therapist Consultation Form and how to use
10. _____ Present secondary targets and explain.
11. _____ Present chain analysis-reasons for their use in team and how to do so.
12. _____ Review group norms.
    - Cell phone clocks are used for the start time of the meeting. The door is closed at the start time.
    - If you are late and the meeting has started, please wait until mindfulness is complete before entering the room.
    - Consultation is limited to three cases at 15 minutes apiece. They are rated as A, B, C and completed in order of need.
13. _____ Review Linehan consultation commitments.
14. _____ Review South Metro DBT Team Additional Consultation Agreements.
15. _____ Obtain commitment to the team.

_____ _____

Signature of Team Member Date

_____ _____

Signature of Team Leader Date

Therapist Consultation Form

Use this form to assist in formulating your thoughts. It is voluntary. Use as needed.

Therapist: Client (initials only):

Therapist questions:

To what extent do you feel judgmental toward the client?
To what extent do you feel judgmental toward yourself?
Are you currently feeling burned out in working with this client and/or this situation?

Therapist Need:

☐ Problem Assessment:
☐ Solution Assessment:
☐ Validation:
☐ Increase Empathy:
☐ What am I looking for from the team?

Client Goals:

Life Threatening:

Therapy Interfering:

Quality of Life:

Secondary Targets:

☐ Emotional Vulnerability ☐ Self-Invalidation

☐ Apparent Competence ☐ Active Passivity

☐ Inhibited Emotional Experiencing/ Inhibited Grieving ☐ Crisis Generating